

Report for [REDACTED]

TEST: Surgical Pathology
Collected Date & Time:

Result Name	Results	Units	Reference Range
Surgical Pathology	SURGICAL P		
SURGICAL PATHOLOGY REPORT			

Patient Name: [REDACTED]
Accession #: [REDACTED]
Med. Rec. #: [REDACTED]
Billing Type: Inpatient
Location: Discharge Location
DOB: (Age:)
Service: Surgery
Gender:
Billing
Taken: Received
Physician(s): [REDACTED] Reported:

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HPAA Discrepancy		X
Prior Malignancy History		X
Local/Synchronous Primary		X
Case in (circle):	QUALIFIED	DISQUALIFIED
Reviewed Initial	W 3/2/12	3/2/12

Specimen(s) Received

- A: Total thyroid
- B: Tip of possible left inferior parathyroid
- C: Level six lymph nodes
- D: Left level two lymph nodes
- E: Left modified radical neck dissection

ICD-O-3
CARCINOMA, papillary,
thyroid
QLF: 826013 Path: 8340/3
Site: thyroid, NOS
C73.9
9-30-12

Pathologic Diagnosis

Total thyroid (A):

- Multifocal papillary carcinoma involving the left thyroid lobe and left portion of thyroid isthmus (largest focus 2.2 cm) with numerous (<10) microscopic foci and remaining thyroid lobe all measuring <1 mm in greatest dimension.
- Tumor confined to the thyroid, negative for extrathyroidal extension.

- Margins of surgical resection negative for tumor involvement.

- Negative for lymphovascular and perineural invasion.
- Background of lymphocytic thyroiditis.

Tip of possible left inferior parathyroid (B):

- Normocellular parathyroid tissue.

Level 6 lymph nodes (C):

- Metastatic papillary carcinoma involving two of three lymph nodes (largest metastatic focus 2.0 cm, negative for extranodal extension).

Level 2 lymph nodes (D):

- Metastatic papillary carcinoma involving one of four lymph nodes (0.5 cm metastatic focus, negative for extranodal extension).

Left modified radical neck dissection (E):

- Metastatic papillary thyroid carcinoma involving two of 23 lymph nodes (2.5 cm and 2.0 cm metastatic foci with extranodal extension present in both nodes).

SURGICAL PATHOLOGY CANCER CASE SUMMARY CHECKLIST: THYROID GLAND RESECTION:

Procedure: Total thyroidectomy.

UUID: 912F801B-814F-4FA4-9216-F62E875AAA7F
TCGA-FY-A3R6-01A-PR

Redacted



Received: In formalin.

Specimen Integrity: Intact.

Specimen Size:

Right lobe: 4.5 x 2.0 x 1.4 cm.

Left lobe: 4.0 x 1.8 x 1.8 cm.

Isthmus: 1.0 x 0.2 x 0.2 cm.

Left neck dissection: 9.0 x 5.5 x 1.3 cm.

Specimen Weight: 14 grams.

Tumor Focality: Multifocal, ipsilateral.

Dominant Tumor:

Tumor Laterality: Left lobe.

Tumor Size: 2.2 cm.

Histologic Type:

Variant: Papillary carcinoma, follicular and classical.

Architecture: Classical and follicular.

Cytomorphology: Classical.

Histologic Grade: Grade 1, well-differentiated margins, uninvolved by carcinoma.

Tumor Capsule: Not present.

Tumor Capsular Invasion: Not applicable.

Lymph-Vascular Invasion: Not identified.

Perineural Invasion: Not identified.

Extrathyroidal Extension: Not identified.

Second Tumor:

Tumor Laterality: Left lobe.

Tumor Size: >10 foci of microscopic tumor surrounding larger nodule, all measuring <1 mm in greatest dimension.

Histologic Type:

Variant: Follicular variant, papillary carcinoma.

Architecture: Follicular.

Histologic Grade: Well-differentiated. Grade 1.

Margins: Uninvolved by carcinoma.

Tumor Capsule: Not present.

Tumor Capsular Invasion: Not applicable.

Lymph-Vascular Invasion: Not present.

Perineural Invasion: Not present.

Extrathyroidal Extension: Not present.

Additional Pathologic Findings: Lymphocytic thyroiditis, benign Hurthle cell nodules and numerous microscopic foci of tumor (>10) present in the left thyroid lobe and focally in the left side of the thyroid isthmus (all measuring <1 mm in greatest dimension).

Ancillary Studies: Not performed.

Clinical History: Defer to clinical correlation as to whether this patient has a history of radiation exposure or family history of thyroid neoplasia.

PATHOLOGIC STAGE: pT2(m), pN1b (number of lymph nodes examined 30, total number of positive lymph nodes 5), pMX

NOTE: Information on pathology stage and the operative procedure is transmitted to this Institution's Cancer Registry as required for accreditation by the Commission on Cancer. Pathology stage is based solely upon the current tissue specimen being evaluated and does not incorporate other relevant data. Pathology stage is only a component to be considered in determining the clinical stage and should not be confused with nor substituted for it. The exact operative procedure is available in the surgeon's operative report.

Primary Pathologist: [REDACTED]

Electronically Signed Out by: [REDACTED]

per TSS, Follicular
variant = 40%

Clinical History
Papillary thyroid cancer.
Intraoperative Consultation

B. Tip of possible left inferior parathyroid: Parathyroid tissue (microscopic evaluation).
[REDACTED]

Gross Description

A. The specimen is received fresh labeled "total thyroidectomy" and consists of a 14 g total thyroid resection. There is a suture designating the pole of the left lobe of the thyroid. The surface is shaggy and gray-purple. The right lobe which is inked blue is 4.5 x 2.0 x 1.4 cm. The left lobe which is inked black is 4.0 x 1.8 x 1.8 cm. The isthmus which is inked yellow is 1.0 x 0.2 x 0.2 cm. The specimen is serially sectioned to reveal a multinodular ill-defined firm tan-brown lesion that encompasses the majority of the left lobe of the thyroid and approximates 2.2 x 1.5 x 1.3 cm. There are additional nonsuspicious well-circumscribed soft tan-brown nodules identified in the isthmus and the right lobe that measure up to 0.4 cm. The majority of the right lobe has a firm red-brown cut surface. Representative sections are submitted as A1-A6 entire left lobe lesion; A7 isthmus and right lobe with nodules; A8 and A9 right lobe with remaining nodules. This specimen is reviewed with [REDACTED]

A portion of the lesion is submitted for [REDACTED] studies.

B. The specimen is received fresh for frozen section labeled "possible tip of left inferior parathyroid" and consists of two irregular tan-pink soft tissue fragments that are 10 mg and average 0.2 cm. They are frozen and submitted in their entirety in cassette B1FS.

C. The specimen is received in formalin labeled "level 6 lymph nodes" and consists of two firm tan-gray grossly positive lymph nodes that are 1.5 and 2.0 cm. Two representative sections from each lymph node are submitted as C1 one lymph node, and C2 other lymph node.

D. The specimen is received in formalin labeled "left level 2 lymph nodes" and consists of four firm tan-gray lymph nodes that range from 0.5 to 2.5 cm. Their cut surfaces are firm tan-gray and grossly positive. They are submitted entirely as D1 one bisected lymph node, D2 two whole lymph nodes; D3 one bisected lymph node.

E. The specimen is received in formalin labeled "left modified radical neck dissection" and consists of a 9.0 x 5.5 x 1.3 cm irregular indurated tan-yellow portion of fibroadipose tissue. There is a double suture designating the lowest level 4 and a single suture designating the lateral level 5. The specimen is divided into a superior half (with level 4) and an inferior half (with level 5). There is no attached salivary gland identified. Identified within the inferior half of the specimen is a prominent vessel that is 1.5 cm in length and 0.4 cm in diameter. There is no gross evidence of tumor involvement. There are two enlarged, grossly positive lymph nodes identified within level 4. There are a few additional firm tan-gray lymph nodes identified that range from 0.2 to 0.6 cm. Also noted separate from the oriented tissue are two irregular, indurated tan-yellow portions
[REDACTED]

of fibroadipose tissue that measure 4 cm in aggregate. Within the tissue there are several firm tan-gray lymph nodes that range from 0.2 to 1.6 cm. Representative sections are submitted in nine cassettes.

Cassette summary: E1 section from one grossly positive lymph node, level 4; E2 section from one grossly positive lymph node, level 1; E3 three whole lymph nodes, level 4; E4 cross-sections of vessel, level 5; E5 two whole lymph nodes, level 5; E6 five whole lymph nodes, level 5; E7 one bisected lymph node and additional tissue; E8 five whole lymph node in additional tissue; E9 five whole lymph nodes in additional tissue.

Microscopic Description

Microscopic examination has been performed on all slides. The pathologic diagnosis encompasses the essential microscopic findings of this case.

Interpretation performed at [REDACTED]
[REDACTED]

[REDACTED]